

RESINSODIO POWDER

Sodium polystyrene sulphonate

FORMULA PER 100 g

Polystyrene sodium sulphonate

(I.N.N.) 99.75 g Excipients (Vanilla essence) q.s.

DESCRIPTION:

White 650 cc jar (multidose container) provided with antitampering stopper containing brown or gilded fine powder, vanillin flavoured. There is also a dosing teaspoon together.

PHARMACOLOGY (SUMMARY OF PHARMACODYNAMICS AND PHARMACOKINETICS)

The cation-exchange resin, sodium polystyrene sulphonate, is not absorbed in the gastro-intestinal tract. Gastrointestinal fluids do not affect it. Thus RESINSODIO® exchanges sodium ions for potassium ions in the colon while the resin is unchanging. The resin is finally excreted in the faeces.

Sodium polystyrene sulphonate is a cation-exchange resin, which exchanges its sodium ions for potassium ions. This exchange occurs in the intestine by an osmotic process. Thus it can be understood that in the organism, via the administration of this resin and its elimination in the feces, it removes the potassium that it carries, until the plasma level reaches that which is believed to be convenient.

The efficacy depends on time and potassium ions concentration.

INDICATIONS

RESINSODIO powder for oral suspension is indicated in the treatment and prevention of hyperkalaemia

CONTRAINDICATIONS

Administration should be stopped when the serum potassium level falls to 5mmol/L. Resinsodio cannot be used in patients with obstructive bowel disease.

- Hypersensitivity to polystyrene sulphonate resins;
- High sodium blood levels;
- Neonates
- Severe hypertension

OVERDOSE AND TREATMENT

In the event of overdosage, the resin should be removed from the alimentary tract by the use of laxatives or enemas and appropriate measures taken to restore potassium levels to normal.

Overdosage may produce serious potassium deficiency and signs and symptoms have to be treated as hypokalaemia.

If any other adverse reaction not previously described is observed, consult your doctor or pharmacist.

WARNING AND PRECAUTIONS

Sodium polystyrene sulfonate may bind orally administered medications, which could decrease their gastrointestinal absorption and lead to reduced efficacy. Administer sodium polystyrene sulfonate at least 3 hours before or 3 hours after other oral medications. Patients with gastroparesis may require a 6-hour separation. (See section Interaction with other medicinal products and other forms of interaction and Posology)

Resinsodio should not be administered orally to neonates and is contra-indicated by any route in neonates with reduced gut motility or in any patient with obstructive bowel disease.

It should also be used cautiously in patients with conditions requiring a restricted sodium intake, such as heart failure and severe hypertension.

Administration of Resinsodio in patients with severe renal damage can result in sodium overloading, and may lead to congestive heart failure. In such cases, calcium polystyrene sulfonate may be used in place of Resinsodio.

Metabolic alkalosis has been reported in patients, particularly those with renal disease, after concomitant oral administration of sodium polystyrene sulfonate and cation-donating antacids and laxatives such as magnesium hydroxide, aluminium hydroxide, or calcium carbonate; the potassium lowering effect will also be diminished.

The possible effects of sodium polystyrene sulfonate on serum electrolytes should be considered when diagnostic measurements are contemplated in patients receiving such treatment.

PREGNANCY AND LACTATION

There are no suitable and controlled study references in pregnant or nursing women that permit to obtain conclusive data. As a result, in these cases RESINSODIO administration should be under closed medical control.

ADVERSE REACTIONS

The adverse reactions observed more frequently appear at the early treatment: anorexia, nausea, constipation (if severe: impaction) and diarrhoea.

INTERACTION WITH OTHER MEDICINAL PRODUCTS AND OTHER FORMS OF INTERACTION

Sodium polystyrene sulfonate has the potential to bind other drugs. In *in vitro* binding studies, sodium polystyrene sulfonate was shown to significantly bind the oral medications (warfarin, metoprolol, phenytoin, furosemide, amlodipine and amoxicillin) that were tested. Binding of sodium polystyrene sulfonate to other oral medications could cause decreased gastrointestinal absorption and loss of efficacy when taken close to the time sodium polystyrene sulfonate is administered. Administer sodium polystyrene sulfonate at least 3 hours before or 3 hours after other oral medications. Patients with gastroparesis may require a 6-hour separation (See section Warning and Precautions and Posology)

The following medicines as they may interact with sodium polystyrene sulfonate and it may therefore be necessary to alter the dose or discontinue some of the treatments:

- Levothyroxine or thyroxine
- Cardiac glycosides (digoxin)
- Cation-donating antacids and laxatives that are not absorbed (magnesium hydroxide, aluminium hydroxide and calcium or aluminium carbonate) because of the risk of metabolic alkalosis

POSODOLOGY

Dose should be prescribed and under medical control, being usually:

Adults:

The recommended dose is 15 g up to four times daily as a suspension in water or syrup or a sweetened paste; administration in sorbitol is recommended to avoid constipation. It should not be given in fruit juices that have high potassium content.

Children:

Acute hyperkalaemia: Recommended dose is up to 1 g/Kg body-weight/day, in divided doses. Maintenance: Recommended dose is 500 mg/Kg bodyweight/ day, in divided doses.

When oral administration is difficult, Resinsodio may be administered rectally as an enema. The usual daily dose is 30g given suspension in 100ml of 2% methylcellulose '450' and 100ml of water and retained, if possible, for at least 9 hours . Following the retention of the enema, the colon should be irrigated to remove the resin.

Initial therapy may continue administration by both oral and rectal routes. Children and neonates can be given rectal doses similar to those suggested by mouth; particular care is needed with rectal administration in children as excessive dosage or inadequate dilution could result in impaction of resin.

Administer sodium polystyrene sulphonate at least 3 hours before or 3 hours after other oral medications. Patients with gastroparesis may require a 6-hour separation. (See section Warning and Precautions and Interaction with other medicinal products and other forms of interaction)

PRESENTATION

It comes in 400 g, multi-dose packs of powder.

STORAGE

Store below 30 °C

MEDICINES SHOULD BE KEPT OUT OF THE REACH AND SIGHT OF CHILDREN.

EXPIRY DATE

5 years from date of manufacture

MANUFACTURER

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PRODUCT REGISTRATION HOLDER

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